PHYSIOTHERAPY & REHABILITATION · CUSTOMER USE CASE

Care that connects. Zonat PhysioSpace.

A step-by-step guide for reception, physiotherapists, nurses, porters, finance, procurement and clinic managers.

Gate to session · Gate to room to discharge

Patient journeys with and without bed accounts.

Therapy materials, aids and machinery procurement.

A complete daily walkthrough for your team.

CUSTOMER WALKTHROUGH · 01

Your clinic, from arrival to follow-up

Open: Start here
Responsible: All staff

1. Zonat PhysioSpace is our example physiotherapy and rehabilitation centre. It receives walk-in and booked patients, delivers outpatient sessions, supports residential rehabilitation, and procures therapy materials and equipment.
2. Choose the journey: outpatient care follows reception → intake → assessment → recommendation (this creates the treatment plan) → finance → session → next appointment. Residential care adds admission from the waiting list → room/bed → ward care → review → discharge.
3. Use the patient record as the common thread. Each handover carries the patient identity, the next action, its owner and the outstanding task.
4. All people, prices and transactions in this guide are training examples. Agree the clinic's actual fees, staff authority and care policies before using them.

Read next	Pages
Roles and setup	3–6
Outpatient journey	7–11
Residential journey and bed accounts	12–18
Procurement and daily walkthrough	19–22
Follow-up and handover	23

Before handing over
A new staff member can identify the next screen and the person responsible.

CUSTOMER WALKTHROUGH · 02

Who does what

Open: Staff & permissions → Roles & permissions
Responsible: Clinic manager

1. Give every staff member their own login and the permissions needed for their duties. The role assignments below are Zonat's working procedure; menu visibility follows the permissions granted.
2. The gate/security team welcomes and directs arrivals. Reception records arrival and departure in the relevant visit or admission workflow. Keep clinical discussions at the care desk.
3. At every handover say: patient, current stage, next action, responsible colleague and any unresolved issue.

Role	Owns the handover
Reception	Identity, booking, check-in, queue and next appointment
Nurse / ward team	Intake, observations, ward tasks, shift handover and leave records
Physiotherapist	Assessment, treatment plan, session notes, progress review and discharge recommendation
Clinical director / authorised approver	Clinical decisions, plan exceptions and discharge approval
Billing / finance	Bills, verified funds, receipts, balances and collections
Porter	Assigned patient movements and completed transfers
Procurement / stock officer	Suppliers, purchases, receipt checks and counted materials
Manager	Permissions, rates, procurement review, reports and daily reconciliation

> **Before handing over**
> Every task has one owner; clinical and financial approvals have named authorities.

CUSTOMER WALKTHROUGH · 03

Make the workspace Zonat PhysioSpace

Open: Administration → Facility
Responsible: Manager / administrator

1. Open Store profile. Set the facility name to Zonat PhysioSpace; enter the clinic's address, telephone, email and genuine contact details. Upload the approved clinic logo.
2. Open Site & branding and check the installation branding and logo used on public and sign-in surfaces. Use the same clinic identity wherever those settings are available.
3. Open Facility settings / Enabled capabilities and select the physiotherapy business profile. Enable the clinic's agreed patient, assessment, plan, session and residential-care capabilities.
4. Check Reception, Patients, Appointments, Physiotherapy, Patient accounts and the ward screens. The facility name should precede the screen title.
5. Preview a patient receipt, session ticket, statement and public booking/portal surface. Verify logo, facility name and contact details. Check desktop, tablet and phone views before staff training.

Branding checklist: sign-in · header · navigation · receipt · statement · session ticket · patient-facing booking · portal · mobile header and footer. This guide illustrates the identity; it does not change a live clinic's settings.

Before handing over
The same approved name and logo appear throughout the training guide and are checked across the clinic's configured screens.

CUSTOMER WALKTHROUGH · 04

Build your service and materials catalogue

Open: Administration → Catalogue & pricing → Services & pricing / Therapy aids & material
Responsible: Manager with stock officer

1. Use the catalogue to separate care services from physical stock. Create the categories Physiotherapy services, Therapy consumables, Rehabilitation aids and Therapy equipment.
2. For each physical item enter a clear name, item code, category, stock unit, purchase cost, applicable issue/selling price and a sensible stock alert. Begin with a verified count or receive stock through a purchase.
3. For each service select the service item type, enter the approved fee and a clear description. A session service is billable care; a pack of electrodes is physical stock.
4. Use one consistent stock unit: bottles for gel, rolls for tape and pieces for electrodes. Record pack conversions only where configured, so receipt quantities match the counted stock.

Record	Example	Unit / purpose
Service	Initial physiotherapy assessment — ₦15,000	One assessment
Service	Rehabilitation session — ₦10,000	One session
Consumable / raw material	Ultrasound gel 1 litre	Bottle; treatment-room supply
Consumable / raw material	Kinesiology tape 5 metres	Roll; treatment supply
Consumable / raw material	Replacement electrode pads	Piece; therapy supply
Rehabilitation aid	Resistance band — medium	Piece; patient aid
Machinery / equipment	Therapeutic ultrasound machine	Unit; clinic-owned equipment
Machinery / equipment	Electrotherapy unit / treatment couch	Unit; clinic-owned equipment

Before handing over
Staff can distinguish billable care, materials used in care, take-home aids and equipment.

CUSTOMER WALKTHROUGH · 05

Agree packages, ward rates and opening records

Open: Clinical configuration → Session packages / Billing & ward policy
Responsible: Manager with finance and clinical lead

1. Create approved session packages from the care services. Our example is six rehabilitation sessions at ₦10,000 each: ₦60,000. The physiotherapist chooses the patient's actual plan.
2. Open Ward → Bed board. Under Add ward / bed, enter Rehabilitation Ward and click Add ward. In the bed form select that ward, enter R01, optionally enter a Rate override, and click Add bed. Repeat for R02. Saving each bed automatically creates its linked bed account; no separate account form is needed.
3. Open Administration → Clinical configuration → Billing & ward policy (also available as Ward → Inpatient billing). Under Daily rates select the code, enter Rate name, Amount and Effective from, then click Save new version. Our example bed rate is ₦20,000 per chargeable day; R02 has a ₦25,000 bed-rate override. Add nursing rates only if the clinic charges them.
4. In Charging policies review the fields: discharge_day_charge = none or full; leave_billing = full, half or none; morning_due / evening_due = task times; auto_settle_wallet = 1 to settle invoices from available funds, or 0 to leave settlement to finance. Confirm admission-day handling with the clinic's billing procedure. Changed policy fields save when you leave the field; save rate versions with the correct effective date.
5. If moving existing patients into the workspace, enter outstanding debt, unused funds and remaining sessions separately in Opening positions, with evidence and an as-of date. A different authorised reviewer approves them.

In Bed payment accounts, tick Enable bed accounts for this workspace to record new transactions by bed. It is off by default; saving a bed still creates its account. Staff need beds_manage to add wards/beds and daily_billing_run to edit billing settings. Patient billing and ward care continue with recording off.

Before handing over
The setup matches the clinic's approved fee sheet and opening records.

CUSTOMER WALKTHROUGH · 06

Gate to reception: identify and register

Open: Clinic → Patients → patient profile
Responsible: Reception

1. Welcome Ada Okoro, who has arrived for outpatient rehabilitation. Ask for her booking or patient reference and direct her to reception.
2. Search Patients before creating a record. Compare name, phone and other identifying details; a shared family phone alone is not enough to identify the person.
3. If she is new, create the patient record with accurate contact and demographic details. Record the required consent and emergency contact through the available patient/document screens.
4. Open her patient profile and confirm the clinical and account links. Use this profile for subsequent care, bills and documents.
5. Check an existing appointment or arrange the appropriate walk-in entry. Explain who will see her next and where to wait.

Gate handover: “Ada Okoro has arrived for her appointment.” Reception confirms the record and creates the digital check-in. The gate is a physical arrival point in this procedure.

Before handing over
One confirmed patient record is handed to the intake team.

CUSTOMER WALKTHROUGH · 07

Reception to intake

Open: Clinic → Appointments / Care queue
Responsible: Reception → nurse

1. For a booked visit, open the appointment and use its check-in action. For a walk-in, open Care queue → Walk-in Check-in, choose the patient and branch, enter a brief arrival note and select Check In.
2. Confirm Ada appears in the queue and the arrival is recorded once. Tell the nurse that her intake is ready.
3. The nurse opens Vitals, records the observations required by the clinic and adds intake notes. Save a draft while completing the form, then finalise the recorded observations.
4. Use Complete Intake to hand the visit to the physiotherapist. The next clinician uses Start Consult when ready.
5. If further intake is needed, return the visit to the nurse using the queue action and explain the reason.

Check-in records arrival. It does not mean a treatment session is completed or that care funds have been spent.

Before handing over
The queue shows the patient's actual stage and the next colleague knows the handover.

CUSTOMER WALKTHROUGH · 08

Assessment to treatment plan

Open: Care queue → visit → recommendation · Patient profile → Treatment Plans
Responsible: Physiotherapist

1. Open the confirmed patient profile and start the applicable assessment form. Record the history, findings, goals and review information required by the clinic's approved template.
2. Attach or review relevant records in Documents & consent and Investigations. Finalise the assessment according to the clinic's process.
3. On the visit, record the recommendation: the chosen service, the number of sessions and the approved amount. Then choose Outpatient or Inpatient for the care setting. Saving the recommendation creates the patient's treatment plan automatically — nothing is typed a second time.
4. For Ada, the recommendation is six rehabilitation sessions at ₦10,000. The plan appears under the Treatment Plans link on her patient record, showing the sessions, goals and care setting. Push the visit to Finance when the assessment is complete.
5. Open the plan to check its billable lines and version. If the course of treatment changes later, use Amend (a new version is kept). A manual New Plan is available on the patient record for care that does not begin with a visit.

Clinical staff decide the appropriate treatment. These examples demonstrate workspace use and do not prescribe a therapy programme.

Before handing over
The plan states what will be delivered, how many sessions and when it will be reviewed.

CUSTOMER WALKTHROUGH · 09

Plan to bill and verified payment

Open: Treatment plan → Create Bill → Patient accounts
Responsible: Billing / finance

1. Open Ada's plan and select Create Bill. Check the patient, service descriptions, quantities and price snapshots before posting the bill.
2. For the six-session example, the bill is ₦60,000. Select Receive payment, record the actual amount, payment type and reference, and provide the patient receipt.
3. For a wallet deposit, open the patient funds view: Cash fund records received cash; Submit transfer evidence records a transfer awaiting verification. Finance verifies against actual receipt of the money.
4. Check debt, available funds, reserved funds, pending evidence and remaining sessions separately. A transfer screenshot awaiting verification is not available care money.
5. If a discount or credit exception is needed, request the appropriate approval. An authorised colleague approves; if the bill changes, check that approval is still current.

Figure	What staff explain
Outstanding debt	The unpaid amount on patient invoices
Available funds	Verified wallet money available for permitted use
Reserved funds	Wallet money held for a specified care purpose
Pending verification	Submitted payment evidence still awaiting review
Sessions remaining	Funded treatment units still to be delivered

Before handing over
The invoice, receipt and session entitlement reflect the actual confirmed payment.

CUSTOMER WALKTHROUGH · 10

Finance to session and next appointment

Open: Treatment plan → Schedule session / Clinic → Physiotherapy
Responsible: Reception → physiotherapist

1. Open the plan’s session entitlements. Confirm there are remaining eligible units, then use Schedule session with the correct date, time and clinician.
2. On arrival, confirm the patient and session. Select Check in, then the physiotherapist selects Start when care actually begins.
3. Record the treatment delivered, response and relevant observations using the clinical notes available to the clinician. Select Complete only when the session has actually been completed.
4. Check that the session is completed and the remaining session count has reduced. For Ada’s fully paid six-session plan, one completed session leaves five.
5. Reception confirms the next appointment and uses Close Visit at the appropriate queue stage. Give the patient the agreed follow-up instructions and direct her to the exit.

For a fully paid package, completing a session uses a session entitlement; it does not collect the same fee again. Record a no-show or cancellation with the matching action, rather than marking care complete.

Before handing over
One visit is closed, one delivered session is recorded and the next appointment is clear.

CUSTOMER WALKTHROUGH · 11

Gate to room: residential admission

Open: Ward → Admissions → Waiting to be admitted
Responsible: Physiotherapist → admissions team

1. The treating physiotherapist decides Bola needs a residential rehabilitation stay during his assessment. His care episode records the decision and he appears in the Waiting to be admitted list at the top of the Admissions screen — no separate request form is needed.
2. Whoever handles admissions opens Admissions and selects Admit this patient on his entry. The admission form fills itself in: the reason is pulled from the physiotherapist's assessment, the bed from the bill where the ward bed was already charged, and the treatment plan from the recommendation.
3. Check each pre-filled detail. Use the date-and-time picker to set when he actually arrived; it opens set to now and only needs rolling back for a stay that began earlier.
4. If Bola still has prepaid sessions, say what happens to them during the stay: continue as normal, pause (they switch back on at discharge), or replace (remaining sessions cancelled and refunded). The question does not appear for patients without prepaid sessions.
5. Check the emergency contact panel beside the form. If the patient record holds no next of kin, open the record and add one before he goes to the ward. Confirm the admission, then the ward team receives the patient and records the first observations and care tasks.

Use the admission overview for room/bed location and its occupancy history. The patient profile continues to connect the stay to assessments, treatment, documents and accounts.

Before handing over
Bola has an active admission, an occupied bed, a named ward team and an understood payment arrangement.

CUSTOMER WALKTHROUGH · 12

A day in the ward

Open: Ward → Nursing tasks / Admissions → open admission
Responsible: Nurse, physiotherapist and ward team

1. At shift start, review active admissions, current beds, outstanding nursing tasks and overdue work. Assign each task to the colleague responsible.
2. Complete observations and nursing tasks only after performing them; add a completion note where needed. Add an ad hoc task when the patient requires additional agreed care.
3. Record observations, handover notes and weekly review notes in the admission. The physiotherapist reviews progress and records the care delivered through the relevant clinical record.
4. Where residential meals are provided, record meal orders and dietary instructions, then mark Provided after delivery.
5. Hand over changes in condition, incomplete tasks, planned sessions, expected leave/transfer and follow-up needs before the next shift.

The nursing task board and porter task board serve different duties. Staff permissions determine which board and actions each colleague sees.

Before handing over
The next shift can see what happened, what remains open and who must act.

CUSTOMER WALKTHROUGH · 13

Moving beds, leave and referral

Open: Admission → Transfer / Temporary leave / External referral
Responsible: Ward team → porter → clinical lead

1. For a bed change, select the available destination bed in the admission's Transfer action and enter the reason. Pass the movement task to the porter.
2. The porter claims and completes the assigned transfer task after the move. Confirm the admission overview and occupancy history now show the actual destination.
3. For authorised temporary leave, record Request leave, approval, actual departure and Return. Check the agreed leave-billing policy and expected return.
4. For an external referral, record destination, reason and urgency. Track departure, procedure progress and return using the referral actions.
5. The clinical team reviews the returning patient and records the review before the next stage of care. Hand over any changed care requirements.

A request alone does not establish that a move has happened. Check the completed movement and occupancy record before explaining the patient's current bed account.

Before handing over
The patient's recorded location and movement status match reality.

CUSTOMER WALKTHROUGH · 14

Residential billing without bed accounts

Open: Ward → Inpatient billing; Patient accounts
Responsible: Finance

1. Leave Enable bed accounts for this workspace switched off. Confirm daily rates and charging policies are configured.
2. Run today’s billing for the correct date, or check the clinic’s scheduled billing run. Review Daily charges on the admission and its linked invoice.
3. Receive payment against the correct patient invoice. If automatic wallet settlement is enabled, review the wallet settlement entries and remaining invoice due.
4. Explain the bill by patient and admission: charge dates, bed/nursing lines, amounts paid and outstanding balance. The Bed board still shows occupancy.
5. At discharge, review final charges and the admission invoice. Record payment or an approved collection arrangement, and keep any unpaid balance visible for finance follow-up.

Switching bed-account recording off stops new assignments; it preserves history already recorded. Patient invoices, funds, receipts and admission charges remain the working financial records.

Before handing over
Ward care and patient billing work without recording new bed-account assignments.

CUSTOMER WALKTHROUGH · 15

Residential billing with bed accounts

Open: Billing & ward policy → Bed payment accounts; Ward → Bed accounts
Responsible: Manager enables; finance reviews

1. Open Administration → Clinical configuration → Billing & ward policy → Bed payment accounts. Tick Enable bed accounts for this workspace; the page reloads after saving. Then open Ward → Bed accounts and select Rehabilitation Ward / R01. Accounts are created automatically when you click Add bed on Bed board. Outpatient transactions use the Outpatient account.
2. Post and review charges through the normal patient/admission billing workflow. Each recorded charge keeps the bed account assigned when posted.
3. Receive payments or record verified wallet funding through the normal patient account screens. Check the payment/deposit account recorded at that time.
4. After a completed transfer, new transactions use the new bed account. Earlier charges and payments keep their original account history.
5. For linked system accounts, open Billing & ward policy and tick Use system accounts for Physio billing. This separate option is off by default. Existing and new beds receive accounts under Accounts; switching off preserves history and stops new routing. On an inpatient plan/bill, Billing account shows the current bed account. On an outpatient plan, choose an active non-bed account before Create Bill; the payment screen uses that choice. Direct payments credit the system account book; wallet settlement does not create another collection. From the bed ledger, authorised staff can Open linked system account book. The patient invoice remains the balance owed.

Bed account display	How to read it
Service charges	Charges attributed to this account
Invoice payments credited	Invoice settlement entries credited here, including wallet settlement
Net money collected	Recorded collection movements, with refunds reducing collection
Charges less invoice payments	Account activity balance; may differ from the patient's invoice due

Before handing over
Finance can explain both bed activity and what the patient still owes.

CUSTOMER WALKTHROUGH · 16

Bed transfer: follow the numbers

Open: Bed accounts → R01 / R02 → linked patient invoice
Responsible: Finance

1. Training example: Bola's posted R01 charge is ₦20,000. He transfers to R02; a later R02 charge is ₦25,000. Assume both are valid chargeable days under the agreed policy.
2. After the transfer, finance receives ₦45,000 against the admission invoice. That new payment credits R02 even though it settles charges from both beds.
3. Open R01: service charges ₦20,000 and invoice payments ₦0 give an account activity balance of ₦20,000.
4. Open R02: service charges ₦25,000 and invoice payments ₦45,000 give an account activity balance of −₦20,000.
5. Open the patient invoice: ₦45,000 billed less ₦45,000 paid gives ₦0 outstanding. Explain this patient balance when discussing whether the bill is settled.

A ₦50,000 verified wallet deposit followed by a ₦45,000 wallet settlement leaves ₦5,000 available, assuming no other holds or movements. The deposit is collected money; the settlement applies that money to the bill. Do not count both as two cash receipts. Earlier transactions without a recorded account are excluded from bed accounts.

Before handing over
Staff understand why a bed account can show a balance while the patient invoice is paid.

CUSTOMER WALKTHROUGH · 17

Room to discharge and gate departure

Open: Admission → Recommend discharge → Discharge
Responsible: Physiotherapist → approver → ward / reception → finance

1. The treating clinician records Recommend discharge with the clinical reason and hands it to the authorised discharge approver.
2. The approver reviews the case and records approval or rejection. Approval requires the discharge reason and summary; record follow-up instructions and the follow-up date where applicable.
3. Finance reviews the admission account and likely final balance. Explain settlement or the agreed outstanding-payment arrangement to the patient or responsible contact.
4. When the patient actually leaves, the authorised colleague selects Record departure. Approval alone does not close the stay. Confirm the admission is discharged and occupancy has closed.
5. Check the final billing result and receipt/statement. Give the patient the discharge summary and confirmed follow-up details through the clinic's approved document process. Reception and the porter coordinate exit and gate handover.

Clinical discharge and financial collection are separate responsibilities. Unpaid invoice balances survive discharge. The final billing run follows recorded departure and applies the configured departure-day policy.

Before handing over
Actual departure is recorded; follow-up is clear and any debt remains visible.

CUSTOMER WALKTHROUGH · 18

Procure materials and machinery

Open: Workspaces → Suppliers → Purchases
Responsible: Procurement officer with manager and finance

1. Create or review the supplier: correct company name, contact, address and opening balance where required. Our training supplier is Zonat Rehab Supplies.
2. Create the purchase with supplier, invoice reference, date, delivery location and catalogue items. Record actual quantities, unit costs, applicable tax and payment terms.
3. Training order: 10 bottles of gel at ₦3,000 = ₦30,000; 20 tape rolls at ₦2,000 = ₦40,000; one ultrasound machine at ₦450,000. Total before any tax or other adjustments: ₦520,000.
4. On receipt, compare the delivery with the supplier invoice, inspect quantities and condition, and save the verified purchase. Review the resulting stock and supplier balance.
5. Finance records the amount actually paid and payment reference. For a short, damaged or returned delivery, record the supported purchase correction/return and keep supplier evidence.

Example part payment: ₦300,000 paid against ₦520,000 leaves ₦220,000 due to the supplier. Therapy income and supplier payments are reviewed separately.

Before handing over
Received stock, the supplier bill and payment records agree.

CUSTOMER WALKTHROUGH · 19

Control treatment materials and equipment

Open: Therapy aids & material → item record; Purchases / Expenses
Responsible: Stock officer with therapy-room lead

1. Count gel, tape, electrodes and other supplies in the same units used in the catalogue. Record the treatment room's agreed replenishment level and review stock alerts.
2. Keep a dated issue/count record of supplies moved into use. Use the authorised stock adjustment process for verified usage, damage or count corrections, with a reason and reviewer.
3. If a patient is billed for a supplied aid, include the actual billable item through the supported billing workflow and check the stock outcome. Keep internal treatment consumption distinct from patient charges.
4. For machinery, retain the purchase invoice, model, serial number, warranty details, location, responsible person and commissioning/service documents in the clinic's equipment records.
5. Plan servicing, log faults and remove unavailable equipment from the working room. Record servicing costs under Expenses and keep the service evidence for manager review.

Recording a completed therapy session is not evidence that gel or tape stock was issued. Check and record physical usage through the clinic's stock procedure. Buying a machine records procurement; the clinic's equipment register holds its operating and maintenance history.

Before handing over
Materials can be counted, and each machine has a responsible custodian and supporting records.

CUSTOMER WALKTHROUGH · 20

Daily walkthrough: open and run the clinic

Open: Reception → Appointments → Care queue → Ward
Responsible: Each team follows its assigned stage

1. 07:30 — Manager: confirm staff cover, service availability and equipment readiness. Stock officer checks room supplies and any expected deliveries.
2. 08:00 — Reception: open the day's appointments and identify booked arrivals. Ward nurse checks Bed board, active admissions, morning tasks and overnight handover.
3. 09:00 — Ada arrives: gate directs → reception confirms patient and checks in → nurse finalises intake → physiotherapist starts consultation, reviews the plan and delivers the scheduled session.
4. 10:00 — Finance checks any bill or payment issue; Ada's funded session is completed once. Reception confirms her next visit and closes today's visit.
5. 11:00 — Bola's ward team records care tasks and observations. If R01 → R02 is approved, the porter completes the movement and the ward confirms the new occupancy.
6. 14:00 — Stock officer checks a supplier delivery against the purchase; finance records the supplier payment. Clinical staff complete afternoon care and progress notes.

Times are an illustrative Zonat workday. Use the clinic's actual shift times and appointment schedule. Check the queue and ward task boards throughout the day so handovers do not remain unattended.

Before handing over
Every patient and delivery has a traceable next step.

CUSTOMER WALKTHROUGH · 21

Daily walkthrough: reconcile and hand over

Open: Patient accounts → Inpatient billing → Clinical reports
Responsible: Finance → manager → next shift

1. 16:00 — Finance verifies submitted payment evidence against receipts, checks pending transfers and reviews today's patient invoices, receipts and available/held funds.
2. Run or verify today's inpatient billing. Review charge dates, bed transfers, leave and recorded departures. Inspect the admission invoice and, if enabled, the bed accounts.
3. Reconcile money by payment method: cash received, bank receipts, wallet deposits, wallet invoice settlements and refunds. Count wallet-funded settlement as application of prior funds.
4. Manager reviews completed sessions, no-shows, remaining care commitments, outstanding invoices, ward occupancy, overdue tasks, supplier dues and stock needing reorder using the relevant registers and reports.
5. Before shift change, list active admissions, changes in condition, unfinished tasks, transfers/leave, next sessions, expected discharges and who must act. The incoming team acknowledges the handover.
6. Check tomorrow's appointments and follow-up commitments. Record actual completion status before logging out of each staff account.

Check	Where to look
Sessions delivered / not attended	Physiotherapy and Clinical reports
Patient debt / paid invoices	Patient accounts and admission account
Verified / held / pending funds	Patient funds and statement
Bed-attributed activity	Bed accounts, when recording is enabled
Supplier dues and received materials	Purchases, Suppliers and catalogue stock
Ward work awaiting action	Nursing tasks, Porter tasks and admission notes

> **Before handing over**
> Clinical work, money, supplier obligations and the next shift's actions reconcile.

CUSTOMER WALKTHROUGH · 22

Follow-up, documents and your first practice run

Open: Patients → profile; Appointments; Documents & consent
Responsible: Reception with clinician and manager

1. Book the next visit from the agreed plan or discharge follow-up. Confirm the date, time, location and contact details with the patient.
2. Keep consent, assessment attachments and discharge documentation under the correct patient record and document category. Where the patient portal is enabled, invite the intended patient through the authorised portal process.
3. Practice Ada's complete outpatient path: identify → check in → intake → assessment/plan → bill/payment → schedule → start/complete → next appointment → close visit.
4. Practice Bola's residential path: physiotherapist recommends admission → Admit from the Waiting-to-be-admitted list → confirm the prefilled reason, bed and treatment plan → ward tasks → transfer R02 → daily bill → discharge recommendation → approval → actual departure → final invoice review.
5. Repeat the financial explanation with bed-account recording off and on. Then practice a gel/tape/equipment purchase and a part payment to its supplier.
6. Ask the trainee to explain who owns each handover and locate the supporting screen. The manager signs off only when the trainee can reconcile the patient invoice and the example transfer accounts.

Quick language: appointment = planned visit; care queue = today's handover stage; plan = agreed care; entitlement = sessions available to deliver; admission = residential stay; bed account = transaction activity assigned to a bed; patient invoice = bill and outstanding balance.

Before handing over
A new colleague can follow the patient journey and explain both billing modes.